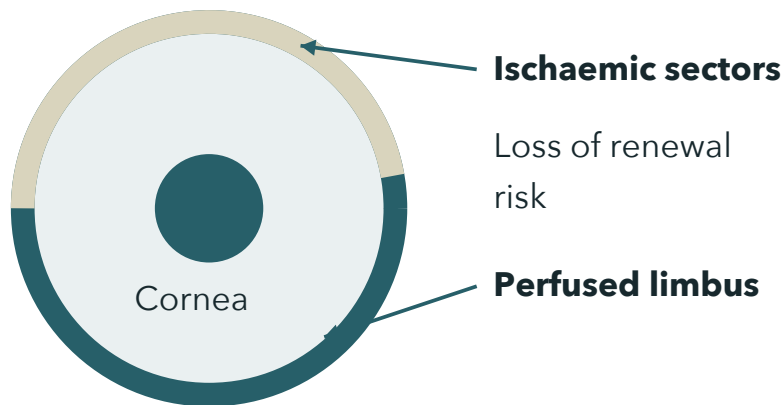


Chemical injury beyond neutral pH

CLINICAL M3-M5 | TYPE B | A focused decision guide

Assess the capacity to heal

The limbus supplies epithelial renewal



Normal pH does not reverse existing stem-cell injury.

Original anterior-eye schematic. Blanched limbal sectors represent ischaemia; they are not a graded patient photograph.

After irrigation

Record	Clinical meaning
Limbal and conjunctival involvement	Stem-cell injury and surface-regeneration risk.
Corneal clarity and epithelial defect	Depth, coverage and visual-axis involvement.
IOP and inflammation	Acute injury and later pressure complications.

Compare injuries fairly

Two eyes with the same pH and defect size may have different prognoses. Extensive limbal ischaemia is a particularly concerning sign; neutral pH confirms chemical removal, not recovery.

References Dua et al.: Chemical injury assessment

Follow the surface recovery

A specialist-directed sequence

- 1 Confirm stable physiological pH and removal of retained particles before the full examination.
- 2 Control inflammation, protect the surface and support epithelial healing using the injury-specific regimen.
- 3 Monitor epithelial closure, stromal thinning, infection and pressure. Escalate nonhealing or progressive melt.

Later finding changes the problem

Finding	Likely consequence
Persistent epithelial defect	Loss of renewal or ongoing surface damage.
Vessels or conjunctiva crossing the cornea	Possible limbal stem-cell deficiency.
Scarring or symblepharon	Optical and mechanical surface impairment.

Reconstruction is staged

Severe injury may need amniotic membrane or later surface reconstruction. A corneal graft alone cannot solve an unstable, stem-cell-deficient surface; decisions are individualised.